



Cotard Syndrome

Cotard Syndrome is a rare psychopathological syndrome characterized by the development of nihilistic delusions that are inconsistent with reality concerning one's own body, existence, or life. Also known as "Walking Corpse Syndrome," this condition may involve more than simply believing that one is dead; the individual may believe that their organs have disappeared, that their body is decomposing, that their body no longer exists, or that they themselves have completely ceased to exist. In some cases, these beliefs may extend beyond the individual's own existence, leading them to believe that the people around them are also dead or no longer exist. Therefore, Cotard Syndrome may be regarded not merely as "believing that one is dead," but rather as a more comprehensive disturbance of the self and the sense of existence.

The syndrome was first described by the French neurologist and psychiatrist Jules Cotard in 1882 as *délire de négation* (delusion of negation). Today, Cotard Syndrome is not recognized as an independent psychiatric diagnosis in the DSM-5 or ICD-11; rather, it is considered a phenomenological syndrome that may occur in the context of various psychiatric, neurological, and medical conditions. Although it is most commonly associated with severe depressive states, it may also occur in association with schizophrenia spectrum disorders, bipolar disorder, delirium, neurocognitive disorders, and certain neurological diseases. Cases accompanied by brain injury, epilepsy, tumors, and other organic brain disorders have also been reported.

The clinical presentation of Cotard Syndrome is highly variable. The most characteristic feature is the presence of nihilistic delusions. The patient may firmly believe statements such as, "I am dead," "My organs are gone," "My heart is not working," "My body no longer exists," or "I have already decomposed." Some patients may also experience unusual bodily sensations, such as the belief that their bodies are burning, disintegrating, or that their internal organs have disappeared. In more advanced cases, the individual may believe

that not only they themselves but also the people around them are dead. For example, in one case report, a patient claimed that the physician in front of them and the other patients in the same room were already dead. This is noteworthy because it demonstrates that the Cotard phenomenon is not limited solely to "seeing oneself as dead."

Cotard Syndrome may frequently occur together with depression, anxiety, guilt, social withdrawal, agitation, hallucinations, and suicidal thoughts. In particular, believing that one is genuinely dead may lead the individual to consider eating or providing self-care unnecessary for sustaining life. Therefore, the syndrome is a serious clinical condition and requires careful assessment with regard to self-harm, neglect of self-care, and suicidal behavior.

The reason why the syndrome develops has not yet been fully elucidated. Current studies suggest that dysfunction within brain networks involving the frontal and temporal regions, parietal areas, the limbic system, and the insula—which play roles in self-perception, body awareness, emotional processing, and reality evaluation—may contribute to its development. Particular attention has been given to a process known as interoception, which enables an individual to perceive their heartbeat, bodily sensations, and other internal signals. According to this view, when an individual fails to experience the expected emotional sense of "aliveness" arising from their own body and internal sensations, they may develop a nihilistic delusion such as "I must be dead" in an attempt to explain this unusual experience. However, these mechanisms have not yet been conclusively established.

There is no single specific treatment for Cotard Syndrome. Treatment is based on identifying the underlying psychiatric or neurological condition associated with the syndrome and addressing it accordingly. Antipsychotics, antidepressants, and other psychiatric treatments may be used when necessary. Particularly in cases involving severe depression and prominent psychotic symptoms, electroconvulsive therapy (ECT) has been reported as an effective treatment option. Nevertheless, when an organic cause or delirium is suspected, neurological and medical evaluation should also be performed without exception.

In conclusion, Cotard Syndrome is an unusual psychopathological condition in which the most fundamental aspects of a person's existential experience -such as "Who am I, does my body really exist, and am I alive?"- may become disrupted. Although a patient's statement, "I am dead," may initially appear to be merely a bizarre thought, it may reflect a complex underlying process involving severe disturbances in body perception, emotional experience, the sense of self, and reality evaluation. Therefore, Cotard Syndrome provides a remarkable clinical window into understanding how the sense of self and the perception of reality are formed, both in psychiatry and in neuroscience.

References:

- 1) Bistas K, Mirza M. Walking Corpse Syndrome: A Case Report of Cotard's Syndrome. *Cureus*. 2024 Jul 4;16(7):e63824. doi: 10.7759/cureus.63824. PMID: 39099940; PMCID: PMC11297383.
- 2) Flach R, Varga JÉ, Herold R, Bányavölgyi V, Osvath P, Fekete S, Voros V, Tényi T. Case Report: Cotard's syndrome associated with suicide attempt-related delirium. *Front Psychiatry*. 2026 Jul 3;17:1890939. doi: 10.3389/fpsyt.2026.1890939. PMID: 42491005; PMCID: PMC13375714.
- 3) Koreki A, Mashima Y, Oda A, Koizumi T, Koyanagi K, Onaya M. You are already dead: Case report of nihilistic delusions regarding others as one representation of Cotard's syndrome. *PCN Rep*. 2023 May 21;2(2):e93. doi: 10.1002/pcn5.93. PMID: 38868142; PMCID: PMC11114400.

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